

HoLEP (Holmium Laser Enucleation)

WARWIKI

Patient Information · A laser surgery for an enlarged prostate of any size

Holmium laser enucleation of the prostate (HoLEP) treats an enlarged prostate (BPH) that blocks urine flow. The surgeon works through a scope in the urethra, so there is **no cut on the skin**. It works for a prostate of **any size**, including very large ones that once needed open surgery.

About This Procedure

Through a scope, a **laser** peels the whole inner prostate (the part causing the blockage) away from its outer shell, like peeling an orange. The freed tissue is pushed into the bladder, cut into small pieces and removed (**morcellation**). The tissue is checked under a microscope. Sometimes a small prostate cancer is found; many of these are slow-growing.

Benefits and Risks in Numbers

5–7 in 10

men have little or no semen afterward (harmless)

1–3 in 100

have urine leakage that lasts

1–3 in 100

later get scar tissue that narrows the urine channel

Numbers are averages from published studies. Your own risk may differ; ask your team.

Why Choose HoLEP

- **Works for any size** prostate, including very large ones
- **Less bleeding than TURP.** It can often be done if you take blood thinners, but the catheter and blood in the urine may last longer.
- Removes the whole blocking part, so the result lasts

It is a special skill. Results are best with an experienced surgeon.

Learn the Terms

HoLEP

Holmium laser enucleation of the prostate.

BPH

An enlarged prostate that is not cancer.

Enucleation

Removing the whole inner prostate from its outer shell.

Holmium laser

The laser that separates tissue and seals small blood vessels.

Morcellation

Cutting the freed tissue into pieces so it can be removed.

Retrograde ejaculation

Semen goes back into the bladder instead of out. It is common and harmless.

Stress leakage

Urine leaks when you cough, lift or stand up.

What changes afterward? Most men get a strong stream that lasts. **Retrograde ejaculation** is common: orgasm feels normal, but little or no fluid comes out. Erections usually stay the same. Some men have urgency or **stress leakage** for weeks to months. This usually gets better, and pelvic floor exercises help.

First day or two

Catheter, usually for a short time. Some men go home the same day.

First few weeks

Pink urine, burning and urgency. Drink fluids. No straining or heavy lifting.

Weeks to months

Any leakage slowly improves. Keep up pelvic floor exercises.

Follow-up visit

Your team checks your flow and the tissue results.

How to Get Ready

- Follow your instructions about eating, drinking and which medicines to stop, including blood thinners.
- A **urine test** checks for infection first.
- Your team may teach you pelvic floor exercises.
- Arrange a ride home.

What Happens

- 1 You get **general or spinal anesthesia**, so you feel nothing, and antibiotics.
- 2 The laser frees the inner prostate. It is cut into pieces and removed.
- 3 A catheter is placed. It usually comes out soon, often in about a day.

Call the office that did your surgery if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Bleeding or burning that gets worse instead of better
- Leakage that is not getting better

Go to an emergency department if:

- You cannot urinate for several hours and your bladder feels full or painful
- Your urine looks like red wine or has clots, or the catheter stops draining
- You cannot reach the office and are worried

Three Things to Remember

1. HoLEP removes the whole blocking part of the prostate through a scope. It works for any size.
2. Little or no semen afterward is common and harmless.
3. Early leakage is common and usually gets better. Pelvic floor exercises help.